

Swelling During Late Pregnancy

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As pregnancy progresses, fluid may accumulate in tissues, usually in the feet, ankles, and lower legs, causing them to swell and appear puffy. Some fluid accumulation during pregnancy is normal, particularly during the 3rd trimester. This condition is called **edema**. Occasionally, the fingers also have mild swelling. If swelling is more than mild in the hands or if there is swelling in the face, the woman should be evaluated by a doctor.

Fluid accumulates during pregnancy because the adrenal glands produce more of the hormones that make the body retain fluids (aldosterone and cortisol). Fluid also accumulates because the enlarging uterus interferes with blood flow from the legs to the heart. As a result, fluid backs up in the veins of the legs and seeps out into the surrounding tissues.

Causes of Swelling in Late Pregnancy

Common causes

Usually during pregnancy, swelling is

- Normal, pregnancy-related edema

Less common causes

Less commonly, swelling during pregnancy results from a disorder (see table

Some Causes and Features of Swelling During Late Pregnancy Such disorders can be serious. They include the following;

- **Preeclampsia**

- [Peripartum cardiomyopathy](#) (heart failure that develops late in pregnancy or after delivery)
- [Deep vein thrombosis](#)

In **preeclampsia**, a disorder that occurs only during pregnancy, blood pressure and protein levels in urine increase. Fluids may accumulate, causing swelling in the face, hands, or feet and additional weight gain; most, but not all, women with preeclampsia have swelling. If severe, preeclampsia can damage organs, such as the brain, kidneys, lungs, or liver, and cause problems in the baby.

In **deep vein thrombosis**, blood clots form in veins located deep within part of the body, often in the legs. Pregnancy increases the risk of this disorder in several ways. During pregnancy, the body produces more of the proteins that help blood clot (clotting factors), probably intended to prevent too much bleeding during childbirth. Also, changes during pregnancy cause blood to back up in veins, making clots more likely to form. If the pregnant woman is less mobile, blood is even more likely to back up in leg veins and clot. The clots may interfere with blood flow. If a blood clot breaks loose, it can travel through the bloodstream to the lungs, blocking blood flow there. This blockage (called [pulmonary embolism](#)) is life threatening.

Peripartum cardiomyopathy is a rare but serious condition that causes shortness of breath and fatigue, as well as swelling.

Risk factors

The risk of preeclampsia, deep vein thrombosis, and peripartum cardiomyopathy are increased by various conditions (risk factors).

High-risk factors for preeclampsia include

- Preeclampsia during a previous pregnancy
- A pregnancy with more than one fetus
- [High blood pressure](#) that was present before pregnancy
- [Diabetes](#)
- Kidney disorders
- [Autoimmune disorders](#)
- [Molar pregnancy](#) (abnormal growth of the placenta with or without a fetus due to an abnormally fertilized egg)

Moderate-risk factors for preeclampsia include

- First pregnancy

- Obesity
- Family history of preeclampsia
- African ancestry
- Lower income
- Age 35 or older
- In vitro fertilization
- Personal history factors (for example, previous infants with low birth weight or small for gestational age, previous adverse pregnancy outcome, more than 10 years between pregnancies)

For **deep vein thrombosis**, risk factors include the following:

- A previous episode of deep vein thrombosis
- Inherited blood clotting disorders
- [Injury to a leg vein](#) that prevents blood from flowing normally
- A disorder that makes blood more likely to clot, such as cancer or kidney or heart problems
- [Cigarette smoking](#)
- Immobility, as may occur after an illness or surgery
- Obesity

For **peripartum cardiomyopathy**, risk factors include the following:

- Age 30 or older
- A previous diagnosis of [cardiomyopathy](#) or other heart problem
- African ancestry
- A pregnancy with more than one fetus
- Preeclampsia
- [High blood pressure](#) that was present before pregnancy

Evaluation of Swelling in Late Pregnancy

Doctors must rule out deep vein thrombosis, preeclampsia, a heart disorder, and other possible causes before they can diagnose normal, physiologic edema.

Warning signs

In pregnant women with swollen legs, the following symptoms are cause for concern:

- Moderate or severe swelling in the hands (hands look noticeably puffy and/or a ring cannot be removed from a finger)
- Swelling that suddenly increases in any part of the body
- Swelling in only one leg or calf, particularly if the area is warm, red, and/or tender or fever is present
- Blood pressure that is 140/90 mm Hg or higher
- Severe persistent headache, changes in vision, confusion, severe upper abdominal pain, or difficulty breathing—symptoms that may be caused by preeclampsia
- Chest pain or difficulty breathing—symptoms that may be caused by deep vein thrombosis

When to see a doctor

Women should go to the hospital immediately if they have any of the warning signs.

Women without warning signs should see a doctor, but a delay of several days is usually not harmful.

What the doctor does

Doctors first ask questions about the swelling and other symptoms and about the medical history. Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause of the swelling and the tests that may need to be done (see table [Some Causes and Features of Swelling During Late Pregnancy](#))

Doctors ask the following:

- When the swelling started
- How long it has been present
- Whether any activity (such as lying on the left side) lessens or worsens it

Doctors also ask about conditions that increase the risk of developing [deep vein thrombosis](#), [preeclampsia](#), and [peripartum cardiomyopathy](#).

Women are asked about other symptoms, which may suggest a cause. They are asked whether they have ever had deep vein thrombosis, [pulmonary embolism](#), preeclampsia, high blood pressure, or heart problems, including cardiomyopathy.

During the physical examination, doctors look for evidence of a serious cause of the swelling. To check for symptoms of preeclampsia, doctors measure blood pressure, listen to the heart and lungs, and may check the woman's reflexes and look at the back of her eyes with an ophthalmoscope (a handheld device that resembles a small flashlight). Doctors also look for areas of swelling, particularly in the legs, hands, and face. Any swollen areas are checked to see if they are red, warm, or tender.



Some Causes and Features of Swelling During Late Pregnancy

Cause	Common Features*	Diagnostic Approach†
Normal (physiologic) edema	Painless swelling (edema) in both legs	A doctor's examination
<u>Preeclampsia</u>	<u>High blood pressure</u> (140/90 mm Hg or higher)	
	Swelling in both feet and ankles and sometimes in the face and/or hands	Measurement of blood pressure
	Sometimes, severe persistent headache, confusion, problems with vision, severe upper abdominal pain, or difficulty breathing	Measurement of protein in the urine Blood tests to check liver and kidney function and a complete blood count
	If very severe, seizures may occur	
	Sometimes risk factors for preeclampsia	
<u>Deep vein thrombosis</u>	Swelling and pain in only one leg or calf	Ultrasonography of the affected leg to check for a blockage in the veins
	Often tenderness, redness, and warmth in the affected area	Sometimes a blood test to measure a substance released from blood clots (called D-dimer)
	If a blood clot travels to the lungs and blocks a blood vessel there (causing <u>pulmonary embolism</u>), chest pain and difficulty breathing	If the woman has chest pain and difficulty breathing, sometimes CT of the



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